NCCP 00655 · VERSION 3A

Your treatment, explained

Stage III NSCLC · consolidation after concurrent chemoradiotherapy · NCCP 00655 v3a · Document revised 4 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Durvalumab 1500 mg every 28 days

28-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact the oncology team promptly for new or worsening cough, breathlessness or chest pain; diarrhoea, blood in stools or significant abdominal pain; yellow skin/eyes or dark urine; reduced urine; a spreading or blistering rash; unusual weakness, severe headache, dizziness, marked thirst or passing much more urine. Severe breathing difficulty, collapse, confusion, seizures, rapidly worsening weakness or difficulty swallowing need emergency assessment. Do not wait for the next infusion.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Open your CUH contact / treatment card

Sources: hse · nccp · smpc

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- Chemoradiotherapy first, then durvalumab
- Your 28-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Chemotherapy and radiotherapy timing
- Tablets, visits and practical planning
- Treatment and its aim
- Tests and symptom review
- Support and urgent symptoms
- Scans and follow-up
- Supportive medicines
- Understanding the evidence
- Questions to ask
- Sources

UNDERSTAND YOUR TREATMENT

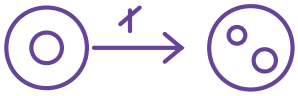
Your immunotherapy medicine

Durvalumab is given after definitive chemoradiotherapy to reduce the chance of lung cancer returning or worsening and improve the chance of longer survival. This is part of a treatment pathway that aims for long-term control and possible cure; it cannot guarantee cure.

PD-L1 IMMUNE CHECKPOINT INHIBITOR

Durvalumab

Intravenous infusion every 28 days



Blocks PD-L1 to support immune recognition of cancer

Durvalumab blocks PD-L1, a protein that can reduce the immune system's ability to attack cancer cells. Removing this signal can help immune cells act against cancer, but the immune system can also inflame healthy organs.

PD-L1 can dampen an immune response → Durvalumab blocks the PD-L1 signal → Immune cells can act against cancer cells

What to watch for

Report new symptoms early and carry the immunotherapy alert card. Do not wait for the next infusion.

Sources: nccp · smpc

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Explore your existing anatomy and side-effect map

THE BIGGER TREATMENT PLAN

Chemoradiotherapy first, then durvalumab

CONSOLIDATION

Durvalumab every 28 days after completed concurrent CRT

After completed concurrent CRT

Every 28 days

Up to 12 months (13 cycles)

Durvalumab is infused on Day 1 of each 28-day cycle. The course lasts up to 12 months (13 cycles), and can finish earlier if the cancer progresses or side effects make continuation unsuitable. The first dose should start within 42 days after chemoradiotherapy finishes.

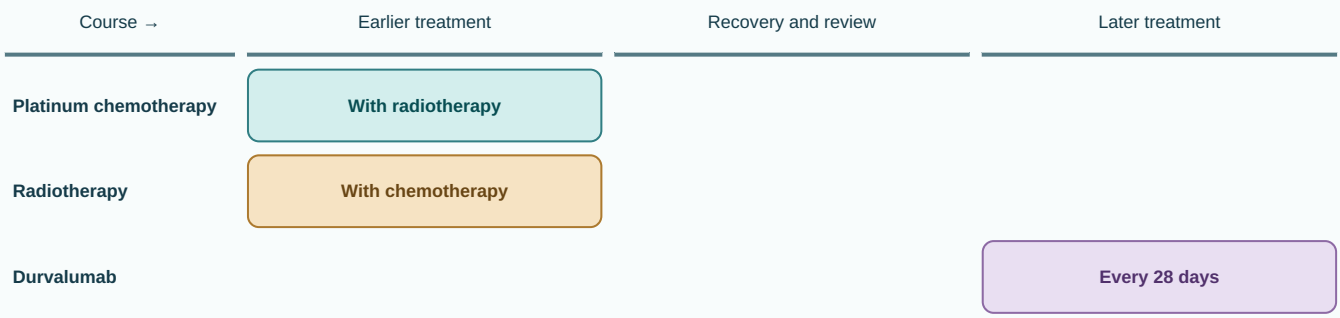
The earlier chemotherapy and radiation course has its own prescribing and support plan. Durvalumab is the consolidation phase covered here.

Sources: nccp

THE WHOLE TREATMENT COURSE

Chemoradiotherapy first, then durvalumab

For this lung-cancer pathway, chemotherapy and radiotherapy are given together. Durvalumab is a separate later phase, after treatment review.



The horizontal axis shows order, not equal time intervals. NCCP specifies starting durvalumab within 42 days after chemoradiotherapy and continuing for up to 12 months (13 cycles), if suitable. The team confirms recovery, no progression and all eligibility requirements.

> **Chemotherapy and radiotherapy overlap in the earlier phase. Durvalumab follows; it is not shown as concurrent with them.**

Educational reference, not a personal appointment calendar. NCCP 00655 v3a, pages 1–2

BASELINE → TREATMENT → FOLLOW-UP

Checks across your durvalumab course

Relative planning stages, not booked appointments or equal time intervals. Follow the team's actual blood-test and scan schedule; extra endocrine, lung or heart tests depend on symptoms and findings.

Check	Before durvalumab	First infusion	Later cycles	Between visits	After completion / a hold
Treatment sequence	Finish concurrent chemotherapy + radiotherapy	Start within the source window	Day 1 every 28 days	No durvalumab tablets at home	Up to 12 months / 13 cycles; review any restart
Blood count / kidney / liver	Baseline tests	Review available results	Regular tests under the team plan	Extra tests for symptoms or abnormalities	Follow abnormalities until resolved / stable
Glucose / thyroid / hormones	Baseline glucose and thyroid tests	Review symptoms and medicines	Regular glucose and thyroid tests	Targeted hormone tests when indicated	Ongoing replacement and follow-up if needed
Lung and other immune symptoms	Review CRT recovery and baseline breathing	Symptom check	Ask before each infusion	Report changes promptly	Delayed immune effects still need assessment
Hepatitis / tumour / scans	Hepatitis B/C screen; validated PD-L1	Confirm stage and no progression	Scans as the consultant plans	New symptoms may need earlier assessment	Agree follow-up and contact arrangements

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00655 v3a · official regimen · EMA Imfinzi product information · sections 4.2–4.6, 5.1

THE WHOLE TREATMENT JOURNEY

Durvalumab 1500 mg every 28 days: treatment and checks

Stage III NSCLC · consolidation after concurrent chemoradiotherapy

Durvalumab is infused on Day 1 of each 28-day cycle. The course lasts up to 12 months (13 cycles), and can finish earlier if the cancer progresses or side effects make continuation unsuitable. The first dose should start within 42 days after chemoradiotherapy finishes.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before durvalumab	First infusion	Later cycles	Between visits	After completion / a hold
Treatment sequence NCCP 00655	Finish concurrent chemotherapy + radiotherapy	Start within the source window	Day 1 every 28 days	No durvalumab tablets at home	Up to 12 months / 13 cycles; review any restart
Blood count / kidney / liver NCCP 00655	Baseline tests	Review available results	Regular tests under the team plan	Extra tests for symptoms or abnormalities	Follow abnormalities until resolved / stable
Glucose / thyroid / hormones NCCP 00655 · SMPC	Baseline glucose and thyroid tests	Review symptoms and medicines	Regular glucose and thyroid tests	Targeted hormone tests when indicated	Ongoing replacement and follow-up if needed
Lung and other immune symptoms NCCP 00655 · SMPC	Review CRT recovery and baseline breathing	Symptom check	Ask before each infusion	Report changes promptly	Delayed immune effects still need assessment
Hepatitis / tumour / scans NCCP 00655	Hepatitis B/C screen; validated PD-L1	Confirm stage and no progression	Scans under the agreed follow-up plan	New symptoms may need earlier assessment	Agree follow-up and contact arrangements

What each check is for

1

Finish CRT and confirm suitability

Before the first durvalumab dose

Chemotherapy and radiation are concurrent first; durvalumab follows after completion.

Ask / act: Review the actual treatment sequence, recovery and baseline tests.

Sources: NCCP 00655

2

Recognise lung and endocrine symptoms

Throughout the course, holds and follow-up

Immune inflammation and radiation effects can overlap; hormone problems can develop with subtle symptoms.

Ask / act: Report changes promptly and keep the alert card available.

Sources: NCCP 00655 · SMPC

3

Complete, pause or stop safely

At toxicity review or the maximum course

The course is capped at 12 months (13 cycles), with earlier stopping if needed.

Ask / act: Agree follow-up, hormone replacement if needed and how to obtain urgent advice after completion.

Sources: NCCP 00655 · SMPC

4

Agree follow-up imaging

After CRT, during treatment and after completion

Look for recurrence when further intervention would be appropriate.

Ask / act: Follow your team’s scan plan and report new symptoms without waiting for a routine scan.

Sources: IMAGING

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-29 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.26

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Durvalumab is infused on Day 1 of each 28-day cycle. The course lasts up to 12 months (13 cycles), and can finish earlier if the cancer progresses or side effects make continuation unsuitable. The first dose should start within 42 days after chemoradiotherapy finishes.

Day of cycle	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	25	26	27	28
Durvalumab · Day 1 IV infusion after review and planned tests	Day 1																											
Symptoms and planned tests	Report new symptoms promptly; follow the team's test schedule																											

Sequence: Concurrent chemotherapy and radiotherapy come first; durvalumab follows after completion.

Start window: The first dose should commence within the source's 42-day window after CRT.

Course limit: Continue no longer than 12 months (13 cycles), with earlier stopping for progression or unacceptable toxicity.

After treatment: Monitoring and reporting continue during holds and after treatment ends.

> **Why these bands stay broad:** This example shows one 28-day durvalumab cycle after concurrent CRT has finished. It is not a dated appointment calendar; the actual prescription and clinical decisions determine visits. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00655 v3a · official regimen ↗

> **There are no guaranteed "safe days".** Use the current prescription and your oncology team's emergency plan. This educational guide does not determine whether a dose can be given.

Read the whole cycle as text

Day 1: Infusion visit

Durvalumab 1500 mg reference dose by IV infusion over about 60 minutes after clinical review and planned tests. Report new symptoms and any treatment changes before the infusion.

Day 2–28: Between infusion visits

No durvalumab tablets are taken at home. Follow the agreed blood-test, supportive-medicine and symptom plan. Report new or worsening symptoms promptly; do not wait for the next cycle.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

Two different parts of treatment

Platinum chemotherapy and radiation are given together first. Durvalumab follows after that course has finished and the team has confirmed that the cancer has not progressed.



STEP 2

Infusion visits

The usual reference dose is 1500 mg, given into a vein over about 60 minutes every 28 days. Allow extra time for the clinical review, tests and preparation. If body weight is 30 kg or less, the team must prescribe an appropriate weight-based dose and interval instead of the fixed 1500 mg dose.



STEP 3

Bloods, scans and transport

Confirm where blood tests are taken, who reviews them and the planned clinic/scan appointments. Allow extra time at the treatment unit and arrange help with travel if fatigue or symptoms make it difficult.



STEP 4

A pause or early finish

Treatment can be paused to investigate symptoms or treat an immune reaction. A missed dose does not automatically extend the course beyond the planned maximum. Restart decisions belong to the consultant team.

Sources: NCCP 00655 v3a · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Immune side effects and the alert card

Immune side effects may start during treatment or after the last dose. Keep the immunotherapy alert card and tell every treating clinician that you have received durvalumab, even during a treatment break or after finishing.

2 Lung symptoms after radiotherapy

Radiation inflammation, immune inflammation, infection and cancer can cause similar symptoms. Report new or worsening breathing symptoms promptly so the team can assess the cause. Do not assume a new cough is only an old radiotherapy effect.

3 Support medicines

Durvalumab alone has minimal sickness risk and no routine premedication is specified. Use anti-sickness or other medicines only as prescribed. Steroids for an immune reaction are a treatment plan supervised by the team, not routine premedication; do not start, reduce or stop them yourself.

4 Hormone replacement and blood tests

Thyroid, adrenal or diabetes problems may need hormone replacement or insulin, sometimes long term. Keep taking prescribed medicines and attend blood tests even when you feel well. Sudden severe weakness, vomiting, faintness, unusual thirst or drowsiness need urgent advice.

5 Pregnancy, fertility and breastfeeding

Discuss pregnancy plans, fertility and breastfeeding before treatment. Effective contraception is advised during durvalumab and for at least three months after the last dose. Durvalumab is not recommended during pregnancy. The team must help decide whether breastfeeding or durvalumab should be stopped or avoided, weighing the benefits and risks.

Sources: nccp · smpc

TREATMENT REFERENCE · 01

Treatment and its aim

Durvalumab is given after definitive chemoradiotherapy to reduce the chance of lung cancer returning or worsening and improve the chance of longer survival. This is part of a treatment pathway that aims for long-term control and possible cure; it cannot guarantee cure.

Who this guide covers

Consolidation for unresectable stage III non-small-cell lung cancer after concurrent platinum chemotherapy and radiotherapy, with no cancer progression and tumour PD-L1 of at least 1%.

Sources: nccp

Sequence and duration

Durvalumab is infused on Day 1 of each 28-day cycle. The course lasts up to 12 months (13 cycles), and can finish earlier if the cancer progresses or side effects make continuation unsuitable. The first dose should start within 42 days after chemoradiotherapy finishes.

Sources: nccp

Infusion and personal decision

The usual reference dose is 1500 mg, given into a vein over about 60 minutes every 28 days. Allow extra time for the clinical review, tests and preparation. If body weight is 30 kg or less, the team must prescribe an appropriate weight-based dose and interval instead of the fixed 1500 mg dose. PACIFIC studied durvalumab every two weeks. The every-four-week schedule is supported by regulatory drug-exposure bridging; PACIFIC did not directly compare these two schedules or test this fixed dose as its randomised treatment.

Sources: nccp · pacific · bridging

TREATMENT REFERENCE · 02

Tests and symptom review

Before starting: blood count, kidney and liver tests, blood sugar, thyroid function, hepatitis B/C screening and the tumour PD-L1 result. Blood count, kidney/liver tests, blood sugar and thyroid tests continue regularly during treatment. The team sets the timing of blood tests and scans and adds tests when symptoms or results need checking.

Breathing after chest radiotherapy

Radiation inflammation, immune inflammation, infection and cancer can cause similar symptoms. Report new or worsening breathing symptoms promptly so the team can assess the cause. Do not assume a new cough is only an old radiotherapy effect.

Sources: nccp · smpc

Hormone monitoring

Thyroid, adrenal or diabetes problems may need hormone replacement or insulin, sometimes long term. Keep taking prescribed medicines and attend blood tests even when you feel well. Sudden severe weakness, vomiting, faintness, unusual thirst or drowsiness need urgent advice.

Sources: nccp · smpc

Scans and targeted tests

Scans follow your consultant's treatment plan. Extra lung, heart, hormone or other tests depend on symptoms and findings; the guide does not set an automatic repeat heart-scan calendar.

Sources: nccp · smpc

TREATMENT REFERENCE · 03

Support and urgent symptoms

Contact the oncology team promptly for new or worsening cough, breathlessness or chest pain; diarrhoea, blood in stools or significant abdominal pain; yellow skin/eyes or dark urine; reduced urine; a spreading or blistering rash; unusual weakness, severe headache, dizziness, marked thirst or passing much more urine. Severe breathing difficulty, collapse, confusion, seizures, rapidly worsening weakness or difficulty swallowing need emergency assessment. Do not wait for the next infusion.

Immune side effects and the alert card

Immune side effects may start during treatment or after the last dose. Keep the immunotherapy alert card and tell every treating clinician that you have received durvalumab, even during a treatment break or after finishing.

Sources: smpc

Lung symptoms after radiotherapy

Radiation inflammation, immune inflammation, infection and cancer can cause similar symptoms. Report new or worsening breathing symptoms promptly so the team can assess the cause. Do not assume a new cough is only an old radiotherapy effect.

Sources: nccp · smpc

Support medicines

Durvalumab alone has minimal sickness risk and no routine premedication is specified. Use anti-sickness or other medicines only as prescribed. Steroids for an immune reaction are a treatment plan supervised by the team, not routine premedication; do not start, reduce or stop them yourself.

Sources: nccp

Hormone replacement and blood tests

Thyroid, adrenal or diabetes problems may need hormone replacement or insulin, sometimes long term. Keep taking prescribed medicines and attend blood tests even when you feel well. Sudden severe weakness, vomiting, faintness, unusual thirst or drowsiness need urgent advice.

Sources: nccp · smpc

Pregnancy, fertility and breastfeeding

Discuss pregnancy plans, fertility and breastfeeding before treatment. Effective contraception is advised during durvalumab and for at least three months after the last dose. Durvalumab is not recommended during pregnancy. The team must help decide whether breastfeeding or durvalumab should be stopped or avoided, weighing the benefits and risks.

Sources: nccp · smpc

TREATMENT REFERENCE · 04

Scans and follow-up

Follow-up continues after treatment finishes.

Agree your scan plan

Published expert guidance suggests chest/abdomen CT with contrast if suitable every 3 months in year 1 after chemoradiotherapy, at least every 6 months in year 2 and at least yearly in years 3–5. Your team agrees your actual plan; this is supplemental guidance, not a fixed NCCP requirement. New symptoms may need earlier assessment. Routine brain scans are not recommended without symptoms after baseline staging.

Sources: imaging · nccp

Why this treatment may be chosen

Durvalumab is given after definitive chemoradiotherapy to reduce the chance of lung cancer returning or worsening and improve the chance of longer survival. This is part of a treatment pathway that aims for long-term control and possible cure; it cannot guarantee cure.

In PACIFIC, durvalumab reduced cancer worsening and improved overall survival compared with placebo after chemoradiotherapy. The results describe groups of trial participants and do not predict what will happen to one person.

Sources: nccp · pacific · bridging

PREPARE FOR THE CONVERSATION

Questions worth asking

Is this more chemotherapy?

Durvalumab is an immunotherapy infusion. It follows the completed chemotherapy and radiation course and works by changing immune signalling.

How long will I receive it?

Durvalumab is infused on Day 1 of each 28-day cycle. The course lasts up to 12 months (13 cycles), and can finish earlier if the cancer progresses or side effects make continuation unsuitable. The first dose should start within 42 days after chemoradiotherapy finishes.

Why report a cough after chest radiotherapy?

Radiation effects, immune inflammation, infection and cancer can look alike. The team needs to assess new or worsening symptoms rather than assume their cause.

Will I need routine steroids before each infusion?

No routine premedication is specified for this regimen. Steroids may be prescribed to treat particular immune side effects, with a supervised dose and taper.

Can side effects happen after I stop?

Immune side effects may start during treatment or after the last dose. Keep the immunotherapy alert card and tell every treating clinician that you have received durvalumab, even during a treatment break or after finishing.

> **Try explaining the plan back:** What is the sequence of chemoradiotherapy and durvalumab, how often are your infusions, what is the planned maximum course, and which symptoms mean you should contact the team immediately? Who should see your immunotherapy alert card?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00655 v3a. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00655 v3a · official regimen
- EMA Imfinzi product information · sections 4.2–4.6, 5.1
- Spigel et al. PACIFIC five-year update, JCO 2022; DOI 10.1200/JCO.21.01308
- EMA durvalumab fixed-dose pharmacokinetic bridging assessment
- HSE · chemotherapy safety and when to contact your team
- Ko et al. 2023 PACIFIC-based expert follow-up imaging recommendations (supplemental; not NCCP-mandated)

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-10-29.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6883/655_V3a_Durvalumab_1500mg.pdf

https://www.ema.europa.eu/en/documents/product-information/imfinzi-epar-product-information_en.pdf

https://pmc.ncbi.nlm.nih.gov/articles/PMC10137068/

https://ascopubs.org/doi/10.1200/JCO.21.01308

https://www.ema.europa.eu/en/documents/variation-report/imfinzi-h-c-4771-ii-0014-g-epar-assessment-report-variation_en.pdf

https://www2.hse.ie/conditions/chemotherapy/side-effects/